جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : مرفت ثابت عبده سليمان
المحافظة : القاهره **الحي / المدينة :** السيده زينب **العنوان :** السيده زينب
رقم البطاقة : 25905270100209 **رقم التليفون :** 01095949233 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 27-05-1959 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

جوستافي روسي

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: مرفت ثابت عبده سليمان
National ID: 25905270100209

GUSTAVE ROUSSY INTERNATIONAL

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Zoledronic acid for bone protection during hormonal therapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



مرفت
ثابت عبده سليمان

١٩ ش يعقوب
السيدة زينب - القاهره

٢٥٩ ٠٥ ٢٧ ٠١ ٠٠٢ ٠٩

LJ7302923

٢٠٩٠٢٠٠١٠٥٢٧٠٢٥٩ ٢٠٢٥/٠٩

بدون عمل

أنثى مسلمة متزوجة

جمال احمد بشير خليل ،

البطاقة سارية حتى ٢٠٣٢/٠٩/٢٧

٧

مبـــادرة رئيس الجمهوريـة
لدعـم صحة المرأة المصرية

04/07/2026

MDT FORM

Name:	Mervat Thabit Abdo Soliman	**MRN:**	190
Age:	67 Y	**ID:**	25905270100209
Menopausal Sta	POST	**Phone:**	01095949233
NRS SCORE:	PS 1	**Marital Status:**	
BMI:		**Family History:**	
CO Morbidity:		**Reason Of MDT:**	

Clinical Data:

BX: IDC

ER: 8/8 **PR:** 8/8 **HER2:** -VE **KI67:**

Sonomammography:

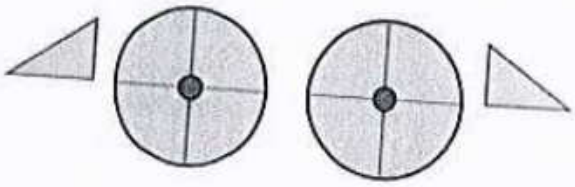
Mets Workup:

History:
POSTM MBC TO LIVER LYMh nodes history rt mrm chemo port tam then 2003 left mrm tam al 2020 liver and lnode mets er8/8 pr8/8 her2-ve rec fasoldex kisqali for 2 y cr then fasoldex ibrance from 7/2023 pet ct although free recently pet ct .pul nodules 9 mm non fgg avid ef 40% cardiac for new ct chest for mdt as approval of ibrance required new mdt from MOH

Recommendations:

MDT Outcome & Plan
Continue ibrance/fasiodex

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
Dr Hanan Ezzat					

[redacted]

Patient Name	: الأستاذه / مرفت ثابت عبده	Request No.	: 13005447
Patient No.	: 25905270100209	Request Date	: 03/08/2023
Path No.	: SR-856/23	Verification Date	: 13/08/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 64 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical data:
Left supraclavicular lesion
Nature of specimen:
Slide revision
Gross examination
1 slides +1 block(5010)
Microscopic examination:
Histological type: Metastatic adenocarcinoma .
Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II
IPT:
GATA3:Positive
Mamoglobulin:Positive
P63:Not reliable
ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)
HER2:No membrane reaction in tumor cells (score 0)
KI67:7% nuclear reaction in tumor cells.
Diagnosis:
Left supraclavicular lesion, slide revision:
Metastatic adenocarcinoma of primary mammry ductal origin, Grade II .
PT:
GATA3:Positive
Mamoglobulin:Positive
P63:Not reliable
ER: Positive (score 8/8).
PR: Positive (score 8/8).
HER2:Negative (score 0)

Verified by :
dr.esraa magdy

Room No. (4) - First Floor Building No. (19) - 9th Corniche - Nile El Qobrasi - Cairo
Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الأستاذه / مرفت ثابت عبده	Request No.	13005447
Patient No.	25905270100209	Request Date	03/08/2023
Path No.	SR-856/23	Verification Date	13/08/2023
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	64 Y	Department	Out Patient
Referreing Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

KI67:7% nuclear reaction in tumor cells.

Prof.Gamal Abdelatty
Dr.Menan Hossam
Dr.Nermin Mohammed

Verified by :
dr.esraa magdy

Room No. (4) - First Floor - Building No. (10) - Public Health Complex (PHC) - Specialized Medical Complex - Nasr City - Cairo Governorate - Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Ministry of Health and Population
National Hepatology & Tropical
Medicine Research Institute

TMRI

وزارة الصحة
الهيئة العامة للمستشفيات والمعاهد
التعليمية
المعهد القومى للأمراض المتوطنة

٦٧٨٣

Patient Name: Karima Essam Ahmed 33Y	Patient ID: 29305220102909
Age: 33 Years, 1 Months, 16 Days	Study Date: 17/02/2026 11:16:07 AM

DYNAMIC MRI STUDY OF THE ABDOMEN

Technique of Examination:

- Axial T1 (in-out phases)
- Axial T2 & T2 fat sat
- Axial DWI & ADC
- Coronal T2
- Multiple Axial post contrast T1 series

MRI Findings:

- Average size of the liver (measures 14.7 cm) showing smooth outline.
- Bilobar scattered small nodules are noted. The largest one measures around 1x1.5 cm at maximum dimensions. All lesions elicit low T1, iso T2 signal intensities, showing post contrast enhancement in the delayed phase .no diffusion restriction.
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Gall bladder is of average distension with no evidence of filling defects or masses inside. CBD is not dilated.
- Patent homogenously enhancing portal vein and its main branches.
- Normal size & MR signal of the spleen, with no focal lesions within its parenchyma.
- Normal site, size, parenchymal thickness & excretory function of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- No ascites.
- No pathologically enlarged pelvi-abdominal lymph nodes.
- Normal MRI appearance of the lung bases.

Left breast mass measures 2x1.5cm , showing delayed enhancement

OPINION:

- **Hepatic bilobar small nodules as described ?hepatic deposits is to be considered in correlation with patient's history for PET scan.**

Left breast mass

Dr. Mira Makram

Dr/zeinab mostafa,MD

دا/زينب

M.R.I
[[DOCTYPE:MRI]]

PET Scan
[DOCTYPEPET_SCAN]

Pat. Name: Merval Sabet Abdou
Procedure Name: PET - CT. Nuclear

Pat. ID: 1811505
Date: 17/02/2026

Follow up Whole body F-18 FDG PET/CT EXAMINATION:

Clinical History:

- *Female patient with history of managed bilateral breast cancer.*
- *PET/CT requested for assessment.*

Technique:

- *For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed simultaneously and used for attenuation correction, anatomical localization and diagnosis.*

Comparison done with PET-CT dated 2-10-2025.

Report

Hepatic reference activity: 2.5

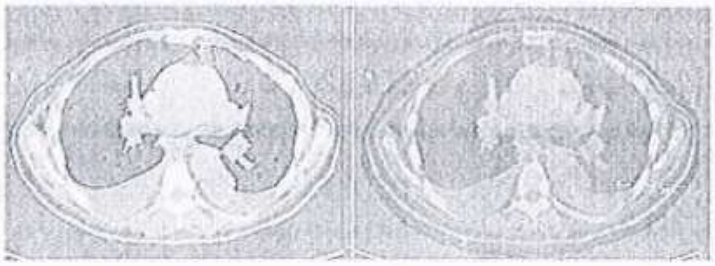
Regions of interest:

- ***Status post bilateral mastectomies; still clear operative bed with no abnormal FDG activity seen to suspect recurrent neoplastic disease.***
- ***No hypermetabolic axillary lymphadenopathy.***

PET Scan
[DOCTYPEPET_SCAN]

Pat. Name: Mervat Sabet Abdou
Procedure Name: PET - CT Nuclear

Pat. ID: 1811505
Date: 17/02/2026



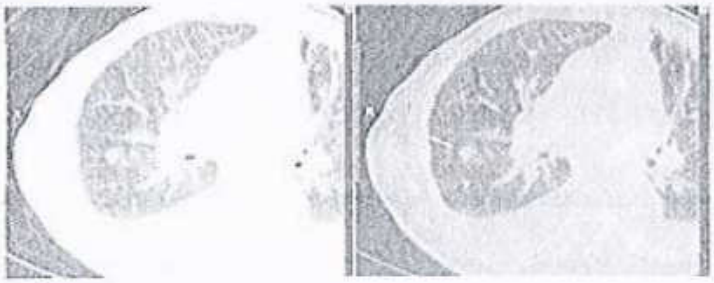
Head and Neck:

* *No hypermetabolic cervical lymphadenopathy could be detected.*
* *Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.*

* *The brain shows uniform tracer distribution with NO hypermetabolic focal lesions.*

Chest:

* *No hypermetabolic axillary or mediastinal lymphadenopathy.*
* ***Mild size progression of the previously seen non-FDG avid bilateral pulmonary nodules, largest measures about 9 mm in the right middle lobe for close follow up.***



* *Few bilateral atelectatic bands.*
* ***Newly developed bilateral pleural effusions.***

PET Scan
[DOCTYPEPET_SCAN]

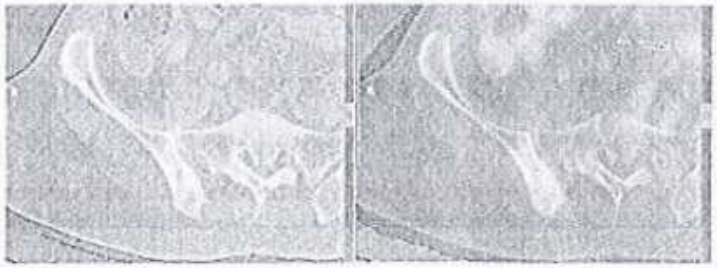
* *There are no active pleural abnormalities.*
* *Physiologic FDG uptake is noted in the heart.*

Abdomen and Pelvis:

* ***No currently seen hypodense hepatic focal lesions detected.***
* ***Calcular gall bladder.***
* *No hypermetabolic abdominal or pelvic lymphadenopathy.*
* *Physiologic diffuse FDG uptake is seen over average size spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.*
* *The pancreas shows normal FDG avidity.*
* *There are no active adrenal nodules.*
* *Physiologic FDG excretion is seen in the kidneys and bladder.*

Musculoskeletal:

* ***Mild regression of the previous FDG avid right iliac bone lytic/sclerotic osseous deposit with SUVmax~2.7.*** *(was SUVmax~4).*



* *Increased FDG activity is seen near the left gleno-humeral articulation, likely of arthritic nature.*

PET Scan
[[DOCTYPE:PET_SCAN]]

Pat. Name: Mervat Sabet Abdou
Procedure Name: PET - CT. Nuclear

Pat. ID: 1811505
Date: 17/02/2026

Conclusion:

- *Status post bilateral mastectomy still showing no definite FDG avid active recurrent operative beds focal lesions detected.*
- *Rather stationary course of the previous FDG avid right iliac bone lytic/sclerotic osseous deposit.*
- *Mild increase in size of the previously seen inactive bilateral pulmonary nodules associated with newly developed bilateral pleural effusions for close follow up.*
- *No newly developed FDG avid lesions detected.*

Thank you for referral

Mahmoud A. Nouh
Nuclear medicine specialist
National Cancer Institute.

Dr Esraa Elkholy , MD
Professor of nuclear medicine
National Cancer Institute

For any inquiries regarding the findings in the report please contact us on

radreport@cairoscan.com.eg

مبـــادرة رئيس الجمهورية
لدعـــم صحة المرأة المصرية

الست المصرية هي صحة مصر

MDT FORM 24/7/2023 G

Name	هرة ثابت عيد	MRN	2739-2023
		ID	26905270100209
Age	62	Phone	01095949233
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	New
CO morbidity			

Clinical Data

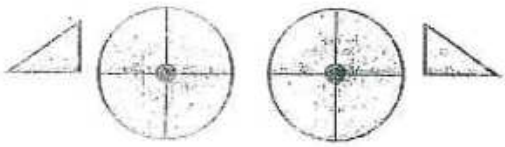
BX: IDC

ER:	PR:	HER 2:	Ki67:
8/8	8/8	-ve	3%

Sonomammography:

Mets Workup:
1997: R+ MRM → chemo → PORT → TAM x 5y
2007: left Brca → Left MRM

R L



TNM

Recommendations: → TAM → AI
2020: LN & liver Mets (+ luminal A)
started Faslodex + Kisqali x 2 years cplan
→ Almost CR by PET

MDT OUTCOME & PLAN Plan: Continue on 2nd line Faslodex + Ibrance after review of IHC

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Patient Name	: الأستاذه / مرفت ثابت عبده	Request No.	: 13005447
Patient No.	: 25905270100209	Request Date	: 03/08/2023
Path No.	: SR-856/23	Verification Date	: 13/08/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 64 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical data:
Left supraclavicular lesion
Nature of specimen:
Slide revision
Gross examination
5 slides +1 block(5010)
Microscopic examination:
Histological type: Metastatic adenocarcinoma .
Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II
IHCPT:
GATA3:Positive
Mamoglobulin:Positive
P63:Not reliable
ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)
PR: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)
HER2:No membrane reaction in tumor cells (score 0)
KI67:7% nuclear reaction in tumor cells.
Diagnosis:
Left supraclavicular lesion, slide revision:
Metastatic adenocarcinoma of primary mammry ductal origin, Grade II .
IHCPT:
GATA3:Positive
Mamoglobulin:Positive
P63:Not reliable
ER: Positive (score 8/8).
PR: Positive (score 8/8).
HER2:Negative (score 0)

Verified by :
dr.esraa magdy

Patient Name	الأستاذه / مرفت ثابت عبده	Request No.	13005447
Patient No.	25905270100209	Request Date	03/08/2023
Path No.	SR-856/23	Verification Date	13/08/2023
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	64 Y	Department	Out Patient
Referring Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

KI67:7% nuclear reaction in tumor cells.

Prof.Gamal Abdelatty
Dr.Menan Hossam
Dr.Nermin Mohammed

Verified by :
dr.esraa magdy

غرفة رقم (4) - الدور الأول - مبنى رقم (707) - شارع المكتبة - ميدان الحصري - القاهرة - مصر
Room No. (4) - First Floor-Building No. 707- Elmaktaba - Mile El Qadeem Cairo, Egypt

مرفت
ثابت عبده سليمان
١٩ ش يعقوب
السيدة زينب - القاهره

٢٥٩٠٥٢٧٠١٠٠٢٠٩

LJ7302923

بدون عمر

انثى مسلمة متزوجة
جمال احمد بشير خليل .
البطاقة سارية حتى ٢٠٣٢/٠٩/٢٧

Name: Mervat Sabet Abdou	Date: 12 Oct 2026
ID: 1811505	Referring:
Exam: PET - CT. Nuclear	

Follow up Whole body F-18 FDG PET/CT

EXAMINATION:

Clinical History:

- *Female patient with history of managed bilateral breast cancer.*
- ***PET/CT requested for assessment.***

Technique:

- *For the aforementioned patient we have done a total body PET scan one hour after IV injection of 370 MBq of 18 F-FDG. MSCT was performed simultaneously and used for attenuation correction, anatomical localization and diagnosis.*

Comparison done with PET-CT dated 17-02-2026.

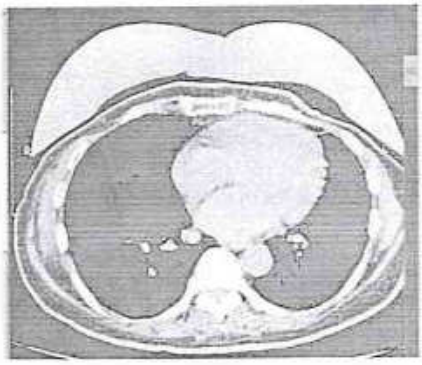
Report

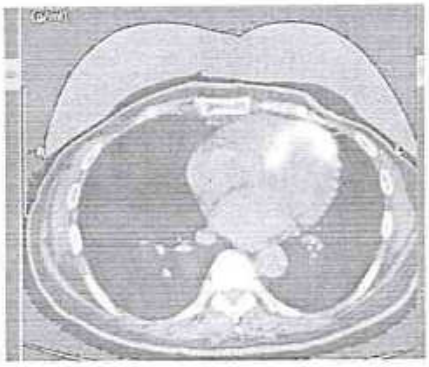
Hepatic reference activity: 2.7(was 2.5)

Regions of interest:

- *Status post bilateral mastectomies; still clear operative bed with no* ***abnormal FDG activity seen to suspect recurrent neoplastic disease.***

Name: Mervat Sabet Abdou	Date: 12 Oct 2026
ID: 1811505	Referring:
Exam: PET - CT. Nuclear	

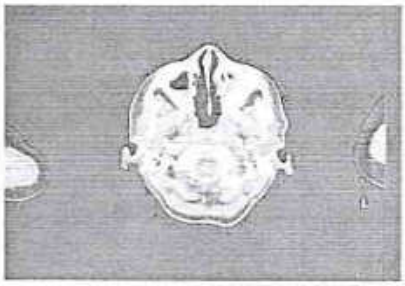


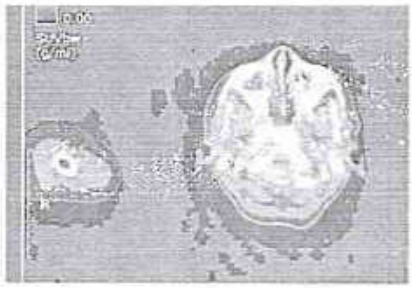


- *Still No hypermetabolic axillary lymphadenopathy.*

Head and Neck:

* *No hypermetabolic cervical lymphadenopathy could be detected.*
* ***Metabolically active FDG avid left sided nasopharyngeal thickening of SUVmax~12.4, likely inflammatory, for clinical correlation and follow up***





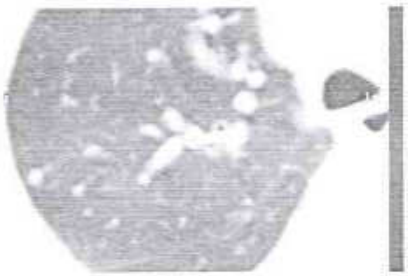
* *Physiologic FDG uptake is seen in the oropharynx, salivary glands, and larynx.*

* *The brain shows uniform tracer distribution with NO hypermetabolic focal lesions*
* *Bilateral maxillary sinusitis .*

Name: Mervat Sabet Abdou	Date. 12 Oct 2026
ID: 1811505	Referring:
Exam: PET - CT. Nuclear	

Chest:

* *No hypermetabolic axillary or mediastinal lymphadenopathy.*
* ***Regression of the previously seen non-FDG avid bilateral pulmonary nodules, largest measures about 4mm(was 9 mm)in the right upper lobe for close follow up.***





* *Few bilateral atelectatic bands.*
* ***Resolution of the old bilateral pleural effusions.***
* *There are no active pleural abnormalities.*
* *Physiologic FDG uptake is noted in the heart.*

Abdomen and Pelvis:

* ***No currently seen hypodense hepatic focal lesions detected.***
* ***Calcular gall bladder.***
* *No hypermetabolic abdominal or pelvic lymphadenopathy.*
* *Physiologic diffuse FDG uptake is seen over average size spleen with homogenous parenchymal texture and no FDG avid focal lesions seen.*
* *The pancreas shows normal FDG avidity.*
* *There are no active adrenal nodules.*
* *Physiologic FDG excretion is seen in the kidneys and bladder.*

Name: Mervat Sabet Abdou	Date: 12 Oct 2026
ID: 1811505	Referring:
Exam: PET - CT. Nuclear	

Musculoskeletal:

* ***Old progressed with newly developed FDG avid ,marrow based , lytic/sclerotic osseous deposits currently seen involving the right iliac bone, left femoral shaft and both humeri with SUVmax~12.3 at right iliac bone .*** *(was SUVmax~2.7).*



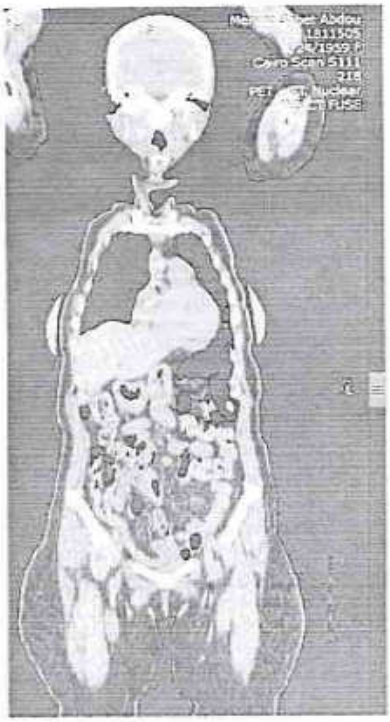




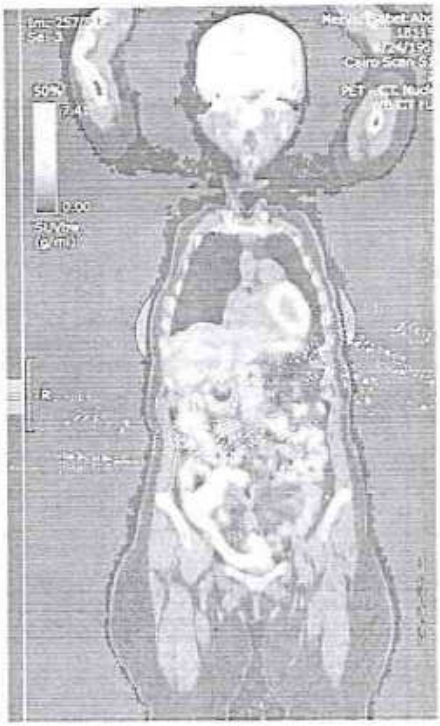
* ***Non FDG avid sclerotic osseous lesion at DV7 for follow up***

* *Still Increased FDG activity is seen near the left gleno-humeral articulation of SUVmax~9, likely of arthritic nature.*

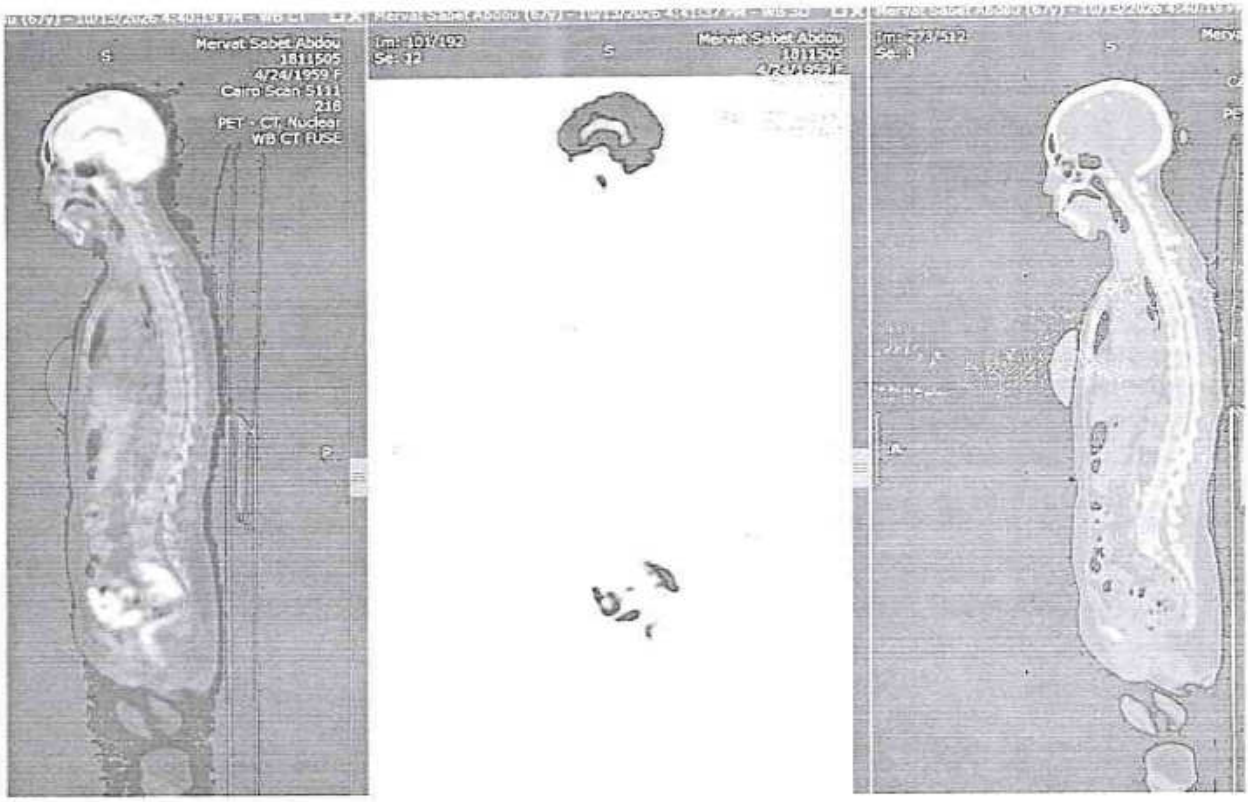
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Exam: PET - CT. Nuclear	







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<u>*Conclusion:*</u>

- *Status post bilateral mastectomy still showing no definite **FDG avid active recurrent operative beds neoplastic lesions detected.***
- *Old progressed and newly developed FDG avid ,marrow **based , lytic/sclerotic osseous deposits***
- ***Regression of the old inactive bilateral pulmonary nodules for follow up.***
- ***Resolution of the old bilateral pleural effusions.***
- ***Metabolically active FDG avid left sided nasopharyngeal thickening, likely inflammatory, for clinical correlation and follow up***

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Thank you for referral

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